



NEWSLETTER

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Dr Harriet Hurrell
Editor in chief



Welcome to the Spring 2026 edition of the **British Heart Valve Society Newsletter**. As the year begins to gather momentum, this issue reflects both transition and progress within our society and the wider field of valvular heart disease. The BHVS continues to grow as a truly multidisciplinary community, bringing together cardiologists, cardiac surgeons, imagers, nurses, scientists and allied professionals who share the goal of improving care for patients with heart valve disease.

This edition marks an important moment for the society as we look back on the contributions of our outgoing President and welcome new leadership. We begin with a message from **Dr Madalina Garbi**, reflecting on her time as President and on the development of BHVS as a collaborative and influential professional society at the centre of patient care. We then look ahead

with an interview with our incoming President, **Dr Dominik Schlosshan**, who shares his perspective on the future of the society and his plans for BHVS in the coming years.

Multidisciplinary care remains fundamental to the management of valvular heart disease. In this issue, Joseph Zacharias offers a thought-provoking viewpoint on Heart Teams and multidisciplinary team meetings (MDTs), exploring how these structures are evolving and whether the time has come to consider more formal approaches to performance and accountability.

Education and collaboration across the entire heart team remain central to the BHVS mission. We are pleased to highlight the programme for the upcoming **BHVS Spring Meeting**, designed specifically for the whole heart team. The meeting will feature two complementary days of learning: the first focusing

on a **core curriculum in heart valve disease**, and the second exploring how ESC guidelines translate into clinical practice, alongside interactive and live MDT sessions that reflect real-world decision-making.

Finally, we conclude with a summary of **recent valve news**, reviewing important clinical trials and developments that are shaping contemporary practice in valvular heart disease.

We hope this edition provides useful insights, stimulates discussion across our multidisciplinary community, and encourages you to join us at the upcoming spring meeting

In addition, if there are any Individuals who would like to contribute to the BHVS newsletter in the future please contact admin@bhvs.org.uk. We would welcome your input.

Harriet

Message from the outgoing president

Dr Madalina Garbi



“It has been an honour to serve as BHVS President. BHVS evolved into a strong multidisciplinary professional society, positioned firmly at the centre of patient care, with a bright future in the hands of the current leadership.”

Madalina Garbi

In conversation with the incoming President

Interview with Dr Dominik Schlosshan - President, British Heart Valve Society

By Vassilios Vassiliou



Vass:

Dominik, congratulations on becoming President of the British Heart Valve Society. To help our readers get to know you, could you tell us a little about your background, where you grew up, where you trained in medicine, and how your career brought you to Leeds?

Dominik:

Thank you. Given my surname, it's a question I'm often asked—particularly as I have lived in Yorkshire for the past 27 years. I was born and raised in Germany and studied medicine in Mainz, a historic university city.

Early in my career I undertook most of my clinical training abroad, including time in Ireland and Australia. My fellowship in Australia was particularly formative, and it is where I developed a strong interest in advanced cardiac imaging, especially structural transoesophageal echocardiography (TOE) and echocardiography more broadly, which has remained central to my clinical practice.

Vass:

And what ultimately led you to practice medicine in the UK?

Dominik:

I was attracted to the structure of UK postgraduate training. I valued the emphasis on bedside teaching and the clarity of training pathways. At the time, bedside teaching was less embedded in Germany, although that may have changed since.

On a personal level, the decisive factor was family. My wife is from Cork in Ireland, and we needed a location that made sense for both of us professionally. Ireland had limited training posts for me at the time, and moving to Germany was not feasible. We were advised to consider the north of England and initially moved to Sheffield, ultimately bringing us to Leeds.

Leeds attracted me as it has always been one of the largest tertiary cardiac centres with ambition and great team ethics.

Vass:

How does life in Leeds compare with Germany, where you grew up?

Dominik:

Leeds has changed enormously since the late 1990s. Culturally, there are a lot of similarities between Germans and British people. Yorkshire and particularly Leeds were always very welcoming to us. I love the ease with which professionalism and light-heartedness coexist.

Leeds is a great city and offers an excellent quality of life, including easy access to countryside such as the Yorkshire Dales.

In conversation with the incoming President

Interview with Dr Dominik Schlosshan - President, British Heart Valve Society

By Vassilios Vassiliou

Vass:

When you're not working, how do you switch off?

Dominik:

Exercise is important for me, particularly running. I also have three daughters and family life has been very full. My two eldest compete in swimming at a high level, so much of our family time has involved supporting training and competition schedules.

When we can, we enjoy simpler things: time together, walking our dog in the Yorkshire Dales, and spending time with my wife.

Vass:

Turning to your clinical career, what initially drew you to cardiology, and later to heart valve disease?

Dominik:

Cardiology appealed to me because it is clinically logical, with well-defined diagnostic pathways and a long tradition of innovation and effective therapies. Cardiovascular disease also affects a large proportion of the population and remains a major cause of morbidity and mortality.

My interest in valve disease developed later. During my fellowship in Australia I became heavily involved in structural TOE and complex valve imaging, and I also encountered a lot of rheumatic valve disease. I came to see that valve disease is common, often under-recognised, and can be managed inconsistently when it is not supported by clear pathways and dedicated services.

Vass:

What do you find most challenging in managing patients with valve disease, and what do you find most rewarding?

Dominik:

The challenges are twofold. First, the scale: valve disease is so common, particularly in older patients. Once you improve access and establish a structured service, you often identify the true volume of unmet need. Balancing demand, workforce capacity, waiting times, and equitable access is an ongoing challenge.

Second, decision-making can be complex, particularly around when to intervene in older patients with multiple comorbidities. These decisions do not always fit neatly into algorithms and require careful multidisciplinary discussion and patient-centred judgement.

The most rewarding aspect has been seeing the improvement in valve care that comes from structured services and genuinely multidisciplinary working. Over the past decade, advances in transcatheter therapies, particularly TAVI, alongside progress in surgery and peri-procedural imaging have transformed what we can offer patients, and have helped raise standards across the whole pathway.

Vass:

Beyond technological advances such as TAVI, are there service-level developments you consider equally important?

Dominik:

Yes! Structured valve services and multidisciplinary valve clinics. These elements of valve care often don't get as much attention, but they can be transformative. Dedicated clinics with appropriate expertise and collaboration between cardiologists, imaging specialists, physiologists and clinical scientists, nurses, surgeons, anaesthetists, and interventional teams improve consistency, surveillance, and timely escalation.

Given the volume of patients, it is also essential to use the workforce intelligently. Not every patient needs to see a doctor at every visit, but every patient needs appropriate monitoring, access to expert review, and timely MDT decision-making when thresholds are reached.

In conversation with the incoming President

Interview with Dr Dominik Schlosshan - President, British Heart Valve Society

By Vassilios Vassiliou

Vass:

What does a typical working week look like for you across clinical work, leadership responsibilities, research, and education?

Dominik:

My NHS practice is focused largely on valve disease, although these patients often have multiple general cardiology problems. I run two valve clinics and spend a significant proportion of my time on structural and peri-procedural imaging, including TOE guidance for surgical and transcatheter interventions and other structural procedures.

I am also involved in MDT work, image review, and advanced imaging lists including cardiac CT, alongside some stress echocardiography.

Leeds is a high-volume tertiary centre with an active programme in valve surgery, transcatheter mitral and tricuspid interventions, and I contribute to clinical service delivery and, where appropriate, trial activity.

My academic background includes an MD focusing on the cardiac effects of biventricular pacing and non-invasive ventilation on cardiac function.

Vass:

As you take on the presidency, what is your overarching vision for the British Heart Valve Society over the next three years?

Dominik:

Although the Society is relatively small, it plays an important role.

I have three top priorities:

Firstly my priority is to strengthen the Society's position as a genuinely multidisciplinary home for all stakeholders involved in valve disease, from early recognition and surveillance pathways through to advanced interventions.

Secondly care for patients with heart valve disease is best delivered in specialist heart valve clinics by healthcare professionals with competency in heart valve disease. The BHVS has previously written a blue print in how this should be delivered but comprehensive heart valve clinics are still underprovided for in the NHS. These require clinical leads in heart valve disease in each hospital with dedicated heart valve nurses and physiologists. A key limiting factor are resources. The BHVS needs to help establish a service framework that obligates hospitals and commissioners to establish appropriate clinic structures and pathways in for heart valve disease in each hospital. Interventions are only the 'tip of the iceberg'. High-quality valve care depends on early diagnosis, appropriate monitoring, timely referral, and effective collaboration between services – the provision of a network-based care of heart valve disease as suggested in the BHVS blue print for heart valve disease.

Lastly the BHVS needs to help establish the 8-week fast track pathway for aortic stenosis treatment as recommended by GIRFT in collaboration with other stake holders such as BCIS and SCTs. This is very important for patient care and has the huge opportunity to save lives and reduce morbidity in patients suffering from unactable delays in treatment pathways in the NHS. Patients deserve better. Delivering this pathway will help lay the foundation for all other key elements for the efficient delivery of effective management of heart valve disease (heart valve clinics with clinical leads and valve nurses and physiologists, effective valve triaging systems, single point of referral system in tertiary centres, rapid access to key imaging diagnostics such as echo and cardiac CT, efficient MDT processing and accelerated surgical or transcatheter treatment)

The Society should continue to champion that whole heart valve disease pathway.

In conversation with the incoming President

Interview with Dr Dominik Schlosshan - President, British Heart Valve Society

By Vassilios Vassiliou

Vass:

What practical steps will you take to deliver that vision?

Dominik:

Education will remain central to achieve this goal. Historically, the Society has focused largely on the annual meeting, but we can expand our educational offer across the year and ensure it meets the needs of the whole multidisciplinary community; physiologists and clinical scientists, nurses, imaging specialists, general cardiologists, and trainees.

Collaboration with key stake holders and national societies involved in the delivery of HVD (BCIS, BSE, PCCS and SCTS but also societies such as the British Society For Heart Failure BSH)) will also be key to achieve this vision.

I would like the Society to continue its role as an important partner in national discussions about pathways and service delivery for valve disease in NHS expert advisory groups, national committees, and national guideline committees building on strong foundations established by previous leadership.

Involving patient groups and collaborating with patient organisations such as heart valve voice will also be a potent tool in establishing the BHVS vision to improve the care for patients with HVD. Patient experience and voices are so important in pushing through key changes to improve the care of HVD.

Vass:

How can we better engage trainees and early-career professionals across the multidisciplinary team?

Dominik:

Supporting training for all health care professionals who will deliver the future care for patients with HVD is so important as it will ensure long-term high-quality care for patients with HVD in the NHS.

Training in heart valve disease is currently poorly defined in training curricula for Cardiology registrars. A Cardiology Registrar requires the skills needed for modern valve care. I think it is important that the BHVS lobby with the SAC for a more structured and prominent training requirements in managing HVD. There is for example currently no structured training program in structural valve intervention or valve specific imaging such as interventional TOE. There is a real need to improve this.

Echocardiography remains fundamental to valve disease, so close collaboration with the British Society of Echocardiography is important, including work on competence and accreditation for physiologists and clinical scientists who are such important contributors to the delivery of care for patients with HVD.

We should also create clearer opportunities for trainees- through webinars, trainee-focused sessions, and meaningful trainee involvement within committees.

In conversation with the incoming President

Interview with Dr Dominik Schlosshan - President, British Heart Valve Society

By Vassilios Vassiliou

Vass:

Which collaborations and partnerships do you see as most important during your presidency?

Dominik:

Nationally, collaboration with all key stake holders involved in the different steps in the pathway for patient with heart valve disease is important. The BHVS is the society that represents all members of the heart team that provides care for patients with HVD.

At primary care level I would like to strengthen our links with the PCCS. We already are in the process of writing a joint statement on frailty.

The British Society of Echocardiography is an important partner and we should also strengthen links with valve nurses, clinical scientists, and related specialties such as heart failure. Strong multidisciplinary relationships are essential if we want to improve pathways.

I am also interested in collaborative work that describes the current national state of valve services. For example, survey-based approaches can help highlight variation in access, waiting times, MDT processes, and treatment timelines—and provide a basis for practical recommendations.

Of course, industry collaboration is important, particularly given the financial realities of running a small professional society. However, collaboration must be structured, transparent, and aligned with the Society's values. We must maintain independence and protect educational and academic integrity. The Society should set its own scientific programme and priorities. At the same time, I would like to explore more stable partnership models that allow the Society to plan confidently rather than relying on year-to-year uncertainty.

Vass:

Looking ahead to the end of your term, how will you measure success?

Dominik:

First, the Society needs to be on a stable financial footing. This can be achieved with strong collaboration with industry.

Second, I would like us to have established a genuinely multidisciplinary education programme that delivers value beyond the annual meeting.

Third, I would like to see a dedicated imaging training day for valve disease, potentially in collaboration with the British Society of Echocardiography, covering both pre-procedural assessment and peri-procedural imaging.

Fourth establishing the fast track aortic stenosis pathway is an important goal with real benefit for patients.

Finally, I would like the Society to have meaningful influence in national discussions on valve pathways, including practical work on improving access and timeliness for patients with left-sided valve disease.

Vass:

What message would you like to share with Society members?

Dominik:

Membership growth is important; a society cannot thrive without an engaged membership base. More broadly, my message is that every professional involved in valve care has a role in improving outcomes for patients with valve disease, and progress depends on multidisciplinary collaboration.

I would encourage members, and potential members, to get involved. The Society should be a practical vehicle for education, standards, and service improvement, and we need contributions from across the community.

I feel a strong sense of responsibility. The Society may not be large, but it is important, and I would like it to be recognised as such. That will only happen through collective effort, not through one individual. If we work together, I believe we can deliver meaningful improvements over the next three years.

Vass: Thank you!

BHVS ACTIVITY

Are “Heart Teams” ready for performance management (Prime time)?

By Joseph Zacharias, Cardiothoracic surgeon

The care of patients with heart valve disease has been greatly improved by the gradual introduction of the idea of heart teams across the country. There are many publications referring to improved mitral valve repair rates and shorter waits for transcatheter aortic valve interventions after the introduction of this concept. There is also wide support for this concept with varying degrees of implementation across hospitals within the NHS. As one of the last few practising cardio-thoracic surgeons in the country, I have the unique opportunity to join weekly lung cancer MDT's and cardiac MDT's within the hospital and the wider cancer and cardiac network. There are some very important differences that have become obvious to me whilst attending these two types of meetings and I would like to highlight some lessons, I believe, the heart teams and cardiac networks could learn from our oncology colleagues.

1: Capturing all patients:

When lung cancer MDT's started in the late 1990's there was a lot of resistance to discussing all patients prior to intervention. Despite the many objections the decision was made that funding for intervention would be linked to the outcome of the MDT. This led to a gradual expansion of the number of patients needed to be discussed but it also led to smoother pathways that shortened discussions for patients with easy decisions freeing time for more complex discussions for the occasional patient. Appointing a chair to run the meeting and sharing the list with radiologists prior to discussions has reduced time for radiology review and other related clinical activity. I feel as much as cardiac patients potentially outnumber lung cancer cases there is an absolute need that every elective cardiac intervention should only be funded if there has been a MDT discussion. There may even be a need for a daily cardiac MDT as already happens in most private hospitals in Europe, to discuss planned referrals with clear pathways for specific valve interventions.

2: Using Scoring Systems for long term survival prediction:

One reason that lung cancer MDT's run so smoothly is the use of the Tumour, Node and Metastasis (TNM) scoring system developed to predict survival in oncology patients. This helped guide interventions and today we are on the 9th version of this staging classification and every new intervention is tested to see if it has resulted to an upward deflection of the survival curve in trials before wider implementation. Cardiac Surgery has led the way in using scoring systems to predict in hospital mortality and some of the scores in use have shown some validity in predicting long term outcomes too. Transcatheter trials have used some of these established cardiac surgery based scoring systems to show non inferior early mortality and reduced early morbidity to standard surgical approaches but we still await real world data regarding the medium to long term outcomes of some of these exciting interventions. As the SYNTAX trial showed using scores can help direct coronary disease patients to one treatment over the other and there is a real need for cardiac valve MDT's to develop and incorporate more scoring systems that predict medium and long term survival in order to understand which patients, despite higher morbidity early, may benefit from

a surgical intervention in the longer term and in which patients will the long term outcomes not be negatively influenced by less invasive interventions. At present there is a tendency to use age as an arbitrary cut off to choose certain interventions but this could be considered ageist and may be a disservice to some very fit and active elderly individuals. Despite having real world data at NICOR for over 15 years on some trans catheter interventions, we have not been able to look more closely into patient characteristics to learn more on this subject from real world experience. We in the UK are fortunate to have access to high level data and there is an urgent need to use AI tools to help triage the right patients to the right interventions.

3: Measuring Performance:

Teams in general only improve when outcomes are measured, compared and discussed in a transparent forum. In cardiac care this measurement can be quite difficult due to the uniqueness of each patient. Despite this there is a need for some form of outcome analysis. The easiest outcome is in hospital morbidity and mortality. The cardiac surgeons in the UK considered themselves to be leading the way in this type of metrics and from 2005 to 2020, all cardiac surgical units were expected to publish

the results of individual surgeon performance and this was championed by many national organisations. Over 15 years the publication of granular data was noticed to have more of a negative impact like risk averse behaviour and poor team dynamics, and so the idea of publication was terminated and cardiac surgeons have gone back to a more team based analysis of data outputs. As data from both the Society for Cardiothoracic Surgeons (SCTS) and the British Cardiac Interventional Society (BCIS) are available online there is an awareness of a variation of practice across the country. Taking the example of TAVI there is a 8:1 ratio of patients being offered TAVI over SAVR in some post codes and in others it is closer to a 1:1 ratio. This variation has cost implications but also may have clinical importance and the only way to understand it would be to publish medium and long term results of one strategy over the other. Coming back to lung cancer teams, there is a yearly output of how many patients get offered surgery or other forms of treatment and apart from 30 day outcomes there is a analysis of 1 year and 5 year results all of which allow comparison of practices across the country with an opportunity to learn from outliers on both ends.

4: Budget sharing:

It is impossible to overlook the budgetary pressure all health care systems are under at present and unfortunately the finances available to healthcare are going to be constrained for many years to come. It is our responsibility to work together with managers and industry to give tax payers the best value for money. There is a wide agreement that good care can also be cheaper care and heart teams will need to see this as an important aspect in deciding the best treatment that has the highest long term benefits for patients. I suspect this is already being considered increasingly within discussions but we do need a way to capture costs that decisions come with as there is likely to be a variation in this aspect too and

there may be opportunities to learn from each other. At present there is a separate budget for different interventions and potentially in future there may be a need for a budget for a pathology like aortic stenosis or mitral regurgitation and the final decision of how the money is spent is left to the individual heart teams with some adjustment for demographics and social deprivation in different post codes. This could then be easily audited to help inform improvements over time.

5: Assessment of new technology introduction:

We are currently in an exciting time of tremendous changes in the world around us and technology appears to be driving many health care improvements. The last few decades have revolutionised the care of heart disease patients with improved heart failure medication, newer transcatheter procedures and advances in 3D endoscopy and robotics. The introduction of these technologies is welcome from a patients perspective but from health care practitioners we need to be mindful that all new technology does not come with a survival or symptom advantage and we have countless examples of this in both medicine and surgery where even harm has followed a rapid unchecked introduction of new devices. We in the UK are fortunate that all interventions are captured in a national registry but we need easier access to this data with a view to analysing this to identify potentially harmful approaches and quickly distill good practice. As already required all patients being considered for these newer technologies need to be discussed in a multi disciplinary team meeting. It is likely that, as in oncology, we will need to widen these discussions to involve more than one MDT and create larger cardiac networks which may involve a hub and spoke model to increase the expertise within these meetings. Once they are set up these networks will need to share budgets and have early and long term results

published on a regular basis so good practice can be identified and replicated.

At present the concept of heart teams is one that is increasingly being adopted across the U.K. There appears to be wide geographical variation in how these teams make decisions looking at the ratio of interventions across different heart teams. There are many lessons that can be learned from our oncology colleagues who have had these structured formats for longer. There will be clinically important benefits to patients and potentially financially better outcomes for the healthcare organisations if we could replicate some of the success that has been achieved in oncology and lung cancer treatment. There is a need for rapid access to diagnostic tests and better pathways once these tests are carried out. The BHVS has a very important role in arguing for this and championing the needs of heart valve patients. Non invasive and imaging cardiologists will be a very important group to lead in this field as they see the patients before any intervention and continue to follow them up after and so are in a very unique position of leading on which patient would benefit from which intervention both for early and long term outcomes. We in the UK need to harness the power of AI and use the wealth of data already captured, to lead in creating scoring systems and better informed pathways for the treatment of patients with heart valve disease in the coming years.

LATEST VALVE NEWS

By Harriet Hurrell

■ 1. Aortic Valve Disease

1.1 PARTNER 3 Trial – 7-Year Outcomes

Valve / Device: SAPIEN 3 TAVI vs SAVR

Population: Low surgical risk patients with severe aortic stenosis

Study Design: Randomized controlled trial (TAVI vs SAVR)

Key Results:

- At 7 years, no significant difference between TAVI and SAVR for:
 - All-cause mortality
 - Disabling stroke
 - Composite major adverse cardiovascular events
- Valve haemodynamics remained stable with **low gradients and similar rates of valve durability between groups.**
- Rates of structural valve deterioration were also low and comparable between groups.

Clinical Implications:

This data supports the **medium-term durability and safety of TAVI** in carefully selected low-risk patients.

<https://www.nejm.org/doi/full/10.1056/NEJMoa2509766>

1.2 ALIGN-AR Trial – TAVI for Symptomatic Aortic Regurgitation Outcomes

Valve / Device: JenaValve Trilogy TAVI

Population: High surgical risk patients with symptomatic moderate-to-severe or severe native aortic regurgitation

Study Design: Prospective, multicenter, single-arm trial

Key Results:

- **Primary safety endpoint (30 days):** 26.2% patients, significantly below the prespecified performance goal.
- **All-cause mortality:** 8.1% at 1 year.
- **Valve performance:**
 - Mean gradient ~4 mmHg at 2 years.
 - Effective orifice area ~2.8 cm².
 - Absent or trace paravalvular leak in ~96% of patients.
- Significant **left ventricular reverse remodelling.**
- Marked improvement in NYHA functional class and quality-of-life scores (KCCQ).
- Pacemaker implantation rates varied by valve size: 16.8% for 23 mm valves, 18.3% for 25 mm valves, and 28.9% for 27 mm valves

Clinical Implications:

ALIGN-AR demonstrates that a **dedicated TAVI system for native AR** is safe and effective, addressing a major unmet need where conventional TAVI devices are suboptimal.

[https://www.thelancet.com/journals/lancet/article/PIIS0140-6736\(25\)02215-9/abstract](https://www.thelancet.com/journals/lancet/article/PIIS0140-6736(25)02215-9/abstract)



Align AR – Jena valve

LATEST VALVE NEWS

By Harriet Hurrell

■ 1. Aortic Valve Disease

1.3 Evolut Low Risk Trial – 7-year outcomes

Valve / Device: Evolut Low Risk Trial (**Medtronic Evolut R or PRO TAVI vs SAVR**)
Population: Low surgical risk patients with symptomatic severe aortic stenosis
Study Design: Randomized controlled trial (TAVI vs SAVR)

Key Results:

- At 7 years, no significant difference between TAVI and SAVR for:
 - ▶ All-cause mortality
 - ▶ Disabling stroke
 - ▶ Composite of death or disabling stroke
- Higher rates with TAVI of:
 - ▶ Permanent pacemaker implantation
 - ▶ Valve reintervention — largely driven by aortic regurgitation (TAVI 9.8% versus SAVR 6.0%)

Clinical Implications:

The 6-year results show no significant difference in survival and disabling stroke between TAVI and surgery in low-risk patients. However, higher reintervention rates—primarily related to aortic regurgitation—remain an important consideration when selecting younger, low-risk patients for TAVI versus SAVR.

<https://pubmed.ncbi.nlm.nih.gov/41697183/>

■ 2. Mitral Valve Disease

2.1 SUMMIT Trial – TMVR with the Tendyne System for Severe Mitral Annular calcification (MAC)

Valve / Device: Tendyne Transcatheter Mitral Valve Replacement (delivered via trans-apical approach)
Population: High-risk patients with severe MAC and mitral regurgitation or stenosis
Study Design: Prospective cohort

Key Results:

- Primary endpoint (freedom from all-cause mortality & HF hospitalization): 60.4% at 12 months
- TMVR in severe MAC is feasible: Technical success achieved in 94.2%
- Acceptable safety profile at 1 year.
- Sustained improvement in heart failure symptoms and quality of life scores (KCCQ).

Clinical Implications:

TMVR with Tendyne offers a potential therapeutic option for patients with severe MAC who are poor surgical candidates.

<https://www.jacc.org/doi/10.1016/j.jacc.2025.10.025>



Summit – Tendyne

LATEST VALVE NEWS

By Harriet Hurrell

■ 2. Mitral Valve Disease

2.2 ENCIRCLE Trial – SAPIEN M3 TMVR for Severe Mitral Regurgitation

Valve / Device: SAPIEN M3 Transseptal TMVR

Population: Patients with severe mitral regurgitation unsuitable for surgery

Study Design: Prospective multicenter study

Key Results:

- **Primary endpoint (1 year):** 25.2% patients, significantly below the prespecified performance goal.
- Positive 1-year safety outcomes.
- Significant reduction in mitral regurgitation which was sustained at 1 year
- Improvement in functional class and quality of life.

Clinical Implications:

Supports expanding the role of **transseptal TMVR** in high-risk MR patients, particularly for those patients not suitable for mitral TEER with anatomy suitable for replacement.

[https://www.thelancet.com/journals/lancet/article/PIIS0140-6736\(25\)02073-2/abstract](https://www.thelancet.com/journals/lancet/article/PIIS0140-6736(25)02073-2/abstract)

■ 3. Tricuspid Valve Disease

3.1 TRILUMINATE Pivotal Trial – 2-year outcomes of TEER for Severe TR

Device: TriClip Transcatheter Edge-to-Edge Repair vs medical therapy

Population: Patients with severe tricuspid regurgitation

Study Design: Pivotal randomized trial

Key Results:

- High procedural safety.
- Significant and sustained reduction in tricuspid regurgitation at 2 years
- Significant improvements in quality of life and functional capacity.
- Decreased rates of heart failure hospitalization compared with medical therapy alone

Clinical Implications:

Tricuspid-TEER is an effective therapy for symptomatic severe tricuspid regurgitation.

<https://www.ahajournals.org/doi/full/10.1161/CIRCULATIONAHA.125.074536>



Encircle trial

LATEST VALVE NEWS

By Harriet Hurrell

3. Tricuspid Valve Disease

3.2 OQUE Transcatheter Tricuspid Valve Replacement

Device: EVOQUE TTVR
Population: Severe tricuspid regurgitation
Study Design: Real-world registry (STS/ACC TVT Registry)

Key Results:

- Low 30-day mortality and heart failure hospitalizations (~3%).
- 97% of patients achieved mild or less TR at 30 days
- Low heart failure hospitalization rates.
- Marked improvement in NYHA class.
- Major bleeding and new pacemaker rates were 1.3% and 14.9%, respectively

Clinical Implications:

Real-world data shows **TTVR is safe and effective**, complementing TEER strategies. These results represent an improvement from the initial trials with lower 30-day mortality and major bleeding events compared to what was observed in TRISCEND II.

<https://eurointervention.pcronline.com/article/five-year-follow-up-of-transcatheter-tricuspid-valve-replacement-with-the-evoque-valve>



Evoque

4. Epidemiology of Valvular Heart Disease

4.1 PREVUE-VALVE Study

Population: A total of 3000 adults aged 65–85 years
Study Design: Community-based echocardiogram screening study

Key Results:

- Moderate or greater valvular disease present in ~8%.
- Any valvular abnormality identified in ~18% (including mild to moderate VHD).
- Tricuspid regurgitation was the most prevalent lesion.
- VHD increases sharply with advanced age, particularly for tricuspid regurgitation and aortic stenosis
- VHD is roughly 20% lower among Black vs. White individuals in the U.S., largely due to lower rates of aortic valve disease.

Clinical Implications:

Highlights the **significant underdiagnosed burden** of valvular heart disease which is projected to grow over the next few decades and supports broader screening strategies.

<https://www.acc.org/latest-in-cardiology/articles/2025/10/24/16/56/mon-2pm-prevue-valve-tct-2025>

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